NCCP 00614 · clinician draft

Carboplatin and vinorelbine · lung cancer

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · carboplatin plus prescribed vinorelbine

Active medicines: Carboplatin + Vinorelbine

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before SACT on Day 1, then daily on Days 2–3
Ondansetron	16 mg orally	Day 1 Adjust for QT/individual risk.
Antiemetic dexamethasone reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	Dedicated carboplatin AUC ≥4 NCIS V 8 row Reference course only: reconcile the aprepitant interaction and any other steroids into one clear prescription.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS row does not specify an end day or PRN qualifier; prescriber/ pharmacy must specify duration and suitability.

Additional phase-specific support

- Confirm prescribed vinorelbine route and review CYP3A4 inhibition by aprepitant; vinca toxicity may increase.
- Use one reconciled antiemetic prescription; do not add a second oral-vinorelbine5HT3agent to Day 1ondansetron.
- Use a constipation-prevention plan and assess bowel symptoms.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00614 v4 · pNCCP 00310 supportive care; NCIS V 8 Table 1 · NCCP NCIS antiemetics V 8

Sources checked 2026-11-02 · SACTCheck v0.84.29 · Regimen-level drafts; prescriber and oncology-pharmacy verification required